ASTEROID.AI

Healthcare B2B Sales Intelligence Report

Outreach Strategy, Ideal Customer Profiles & Market Positioning

Primary Market: United States | Focus: Mid-Market & Enterprise | Approach: Consultative Sales

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1. Executive Summary

This report provides a comprehensive sales intelligence analysis for Asteroid.ai, focused on identifying, targeting, and converting high-value B2B healthcare clients in the United States, with secondary attention to select international markets (UK, India, Gulf states). The analysis is grounded in Asteroid's actual product capabilities, existing customer deployments, and validated use cases.

Asteroid.ai occupies a unique position in healthcare automation: it enables organizations to automate any browser-based or legacy-system workflow without requiring APIs. In an industry where the vast majority of operational systems—EHRs, payer portals, clinical trial management systems, fax platforms—lack modern integration points, this is a transformative capability.

Key Strategic Findings

- **Total addressable pain:** Healthcare organizations in the US spend an estimated $40B+ annually on administrative labor tied to manual, browser-based operations—prior authorization, coverage verification, claims status, and data entry across fragmented portals.
- **Asteroid's wedge:** No-API browser agents that work across legacy EHRs, payer portals, CTMS, and Citrix environments. Competitors require APIs that do not exist, or build fragile RPA scripts that break when portals change.
- **Primary ICP:** Mid-market healthcare operations companies (Series A+, 50–500 employees) with large ops teams performing 100s–1,000s of repetitive browser tasks daily. Secondary: enterprise payer services and RCM firms.
- **Fastest path to revenue:** Prior authorization and payer portal automation—massive pain, active outbound already underway, clear ROI story (3–10x labor cost reduction).
- **Recommended deal size:** $6K–$50K+/month for mid-market; $50K–$200K+/month for enterprise. Current customer range validates $3K–$6K/month as entry, with expansion potential.

Recommendations at a Glance

Priority	Action	Expected Outcome
1	Double down on prior auth outbound campaign (30+ leads)	5–10 qualified demos in 30 days
2	Build mid-market playbook around payer portal automation	Repeatable $6K–$15K/mo deals
3	Develop Thyme Care as anchor case study for enterprise	Credibility for 50–250 employee ops teams
4	Launch Citrix/CUA positioning for hospice & home health	Differentiated pipeline with zero competition

Priority	Action	Expected Outcome
5	Create voice-to-browser content targeting AI voice cos	Inbound pipeline from digital health startups

2. Asteroid Product Breakdown

Asteroid.ai is an AI-powered automation platform purpose-built for workflows that run in browsers and legacy desktop applications. Unlike traditional integration platforms that require APIs, Asteroid's agents interact with systems the same way humans do—by navigating web interfaces, filling forms, clicking buttons, and reading screens.

Core Agent Types

Browser Agents

Asteroid's browser agents operate directly within web-based applications. They can log into payer portals, navigate EHR interfaces, submit forms, upload documents, and extract data—all through the browser, exactly as a human operator would. These agents handle the majority of healthcare automation use cases: payer portal automation, coverage verification, claims status checks, fax workflows, and portal updates.

- **Production proof point:** Thyme Care runs 1,000s of daily browser agent executions across portals and Documo for fax workflows, with enterprise-grade reliability.
- **Self-serve proof point:** Vitable Health's VP of Business Operations (non-technical) independently builds agents across 10+ payer carrier portals using Asteroid's no-code builder, Astro.

Computer-Use Agents (CUA)

For systems that don't have web interfaces at all—legacy desktop applications accessed through Citrix virtual desktops or remote sessions—Asteroid deploys computer-use agents. These agents see and interact with the screen at the pixel level, navigating desktop applications that have no browser or API access whatsoever.

- **Production proof point:** Moonset Health uses CUA agents to navigate 10+ legacy hospice EMR systems across the US via Citrix. This is Asteroid's first and most advanced desktop agent deployment, and a capability competitors cannot match.

Voice-to-Browser Pipeline

Asteroid integrates with AI voice agent platforms to create end-to-end automation pipelines. A voice agent completes a phone call (intake, scheduling, pre-screening), and the call transcript is automatically passed to an Asteroid browser agent via API. The browser agent then executes the required actions in the target system—booking an appointment, filling a form, updating a record.

- **Production proof point:** InTouchNow (UK-based AI receptionist for GP practices) uses this pipeline to automate GP appointment booking in NHS and private systems 24/7. Delfa uses it for clinical trial patient enrollment—voice agent pre-screens patients, then Asteroid fills out CTMS screening questionnaires automatically.

Key Differentiators

Differentiator	What It Means	Why It Matters in Healthcare
No APIs Required	Agents interact via browser/screen, not code integrations	Most EHRs, payer portals, and CTMS lack APIs entirely; this is the only way to automate them
HIPAA Compliant	Encrypted infrastructure, full audit trails on every execution	Hard requirement for any healthcare automation; eliminates compliance risk objection
Minutes to Production	Agents built with Astro no-code builder, not months of engineering	Delfa went from zero to production in 30 minutes; radically faster than RPA or in-house builds
Self-Healing Agents	Agents detect portal changes and adapt without manual reconfiguration	Payer portals change frequently; traditional RPA breaks, Asteroid adapts
Computer-Use (Citrix)	First healthcare-deployed CUA for legacy desktop systems	Opens entire hospice, home health, and legacy EMR market that nobody else can touch
Human-in-the-Loop	Guardrails, evaluations, and human oversight built into every workflow	Critical for clinical and compliance-heavy healthcare operations
Works Across Any Portal	Not locked to specific EHRs or payers	Vitable builds across 10+ carrier portals; scales horizontally without per-integration cost

3. Healthcare Market Analysis

The US healthcare system is uniquely fragmented. There are 1,000+ EHR vendors, hundreds of payer portals, dozens of CTMS platforms, and countless government and state-specific systems. The vast majority of these systems were built before the API era and offer no programmatic access. This creates an enormous operational burden: healthcare organizations employ thousands of staff to manually log into portals, copy data, fill forms, check statuses, and upload documents—tasks that are repetitive, high-volume, error-prone, and compliance-sensitive.

Market Segments

Segment 1: Healthcare Operations Companies

These are established healthtech companies (typically Series A and beyond, 50–500+ employees) with large operations teams performing repetitive browser-based work at high volume. They represent Asteroid's highest-value segment due to deal size, urgency, and operational scale.

Typical workflows: Portal updates, fax sending and receiving, prior authorization submissions, coverage verification across dozens of payer portals, claims status checks, patient record updates in EHRs.

Operational bottlenecks: Ops teams of 20–200+ people spending 60–80% of their time on repetitive browser tasks. Each staff member handles only 30–50 tasks per day due to portal complexity. Error rates of 5–15% on manual data entry. Scaling requires linear headcount increases.

Where Asteroid fits: Replace or augment ops staff on high-volume, repetitive portal work. Agents run 24/7, handle 1,000s of tasks daily (as proven by Thyme Care), and maintain full audit trails. A single Asteroid deployment can replace 5–20 FTEs worth of manual labor.

Why current solutions fail: Portals lack APIs, so traditional integration is impossible. RPA tools (UiPath, Automation Anywhere) require extensive scripting, break when portals change their UI, and lack healthcare-specific compliance features. In-house automation teams spend months building and then maintaining fragile scripts.

Reference customer: Thyme Care ($82M raised, oncology navigation, Nashville)—$3,000/mo, 1,000s of daily executions, browser agents across portals and Documo.

Segment 2: Prior Auth & Payer Portal Automation Companies

This segment includes any organization struggling with prior authorization workflows across browser-based payer portals. It spans RCM companies, health systems, specialty care providers, and dedicated prior auth firms. Asteroid is actively validating this use case with outbound to 30+ leads.

Typical workflows: Log into payer portal, navigate to prior auth section, fill multi-page form with patient demographics, diagnosis codes, procedure codes, and clinical justification, upload supporting clinical documentation (lab results, physician notes), submit, then check back

repeatedly for approval status. Repeat across 10–50+ different payer portals, each with a different interface.

Operational bottlenecks: Each prior auth takes 15–45 minutes manually. Organizations process 50–1,000+ per day. Staff must learn dozens of different portal interfaces. Denial rates of 10–25% require rework and appeals. Portals change their UI without notice, breaking any existing automation.

Where Asteroid fits: Automate the entire prior auth submission, document upload, and status tracking workflow across any payer portal. Self-healing agents adapt when portals change. Full audit trail satisfies payer and compliance requirements.

Why current solutions fail: Payer portals are the quintessential no-API environment. They are browser-based, frequently change, and are deliberately designed to resist programmatic access. API-based solutions like CoverMyMeds only cover a fraction of payers. RPA breaks constantly. Manual labor is the default.

Market sizing: Prior auth processing alone costs the US healthcare system an estimated $31B+ annually in administrative labor. Even capturing 0.1% of this market represents a $31M revenue opportunity.

Segment 3: Digital Health Startups

Startups building vertical AI products for healthcare that need to integrate with legacy systems (EHRs, payer portals, CTMS) as part of their core product. They are smart buyers who understand AI limits, move fast, and have small engineering teams. Voice agent companies are a particularly strong sub-persona.

Typical workflows: Patient data retrieval from EHRs, automated form completion in CTMS, appointment booking in NHS/private systems, document upload and extraction across healthcare portals.

Operational bottlenecks: Small eng teams (3–15 people) cannot afford to build and maintain integrations with dozens of legacy systems. Each integration can take 2–6 months to build in-house. Legacy systems change without notice, breaking custom code.

Where Asteroid fits: Provide the “last mile” integration layer. Startups use Asteroid’s API to programmatically trigger browser agents that interact with legacy systems on their behalf. Saves 95% of engineering effort (as demonstrated by Delfa).

Reference customers: Delfa (clinical trial enrollment, 30 min to production), InTouchNow (UK GP booking, voice-to-browser), Moonset Health (hospice EMR, Citrix/CUA).

Secondary International Markets

- **United Kingdom:** NHS systems for GP appointment booking (InTouchNow validates this). Large market with legacy IT infrastructure. Voice-to-browser pipeline is directly applicable.
- **India:** Rapidly growing healthtech sector. Hospital chains running on legacy systems. Government health portals (ABHA, CGHS) with manual workflows. Cost arbitrage makes even modest automation savings compelling.
- **Gulf States (UAE, Saudi):** Major healthcare digitization investments. Legacy systems being modernized. High willingness to pay for automation. Compliance requirements growing.

4. Ideal Customer Profiles

Profile Qualification Framework

Signal	Green Flag (Perfect Fit)	Red Flag (Bad Fit)
Workflow type	Repetitive, browser-based portal work (data entry, form filling, status checks)	One-off analytical tasks, complex clinical decision-making
System landscape	Legacy EHRs, payer portals, CTMS with no APIs	Modern systems with robust APIs and existing integrations
Volume	100s–1,000s of tasks per day	Fewer than 20 tasks/day (hard to justify ROI)
Team size	Ops teams of 10+ doing manual portal work	Fully automated operations with no manual component
Compliance needs	HIPAA required, audit trails critical	No compliance sensitivity (less differentiation for Asteroid)
Integration attempts	Tried RPA/in-house and it broke, or spending heavily on manual labor	Happy with current automation or no interest in change
Buying capacity	$3K–$50K+/mo budget for automation	Sub-$1K budget, extreme cost sensitivity
Decision maker access	Head of Ops, VP Ops, CTO, or Rev Cycle Manager reachable	No clear owner of ops automation

Primary ICP: Mid-Market Healthcare Ops

Attribute	Detail
Company size	50–500 employees
Funding stage	Series A through Series D, or profitable
Revenue	$5M–$100M ARR
Ops team	20–200+ staff doing manual browser work
Daily task volume	500–5,000+ portal interactions per day
Systems used	3–20+ different payer portals, 1–5 EHRs, fax platforms
Pain intensity	Ops labor is 30–60% of total operating cost
Buying persona	Head of Operations, VP Operations, VP Engineering
Deal size	$6K–$50K/month
Sales cycle	4–8 weeks (pilot to production)

Secondary ICP: Enterprise Payer Services & RCM

Attribute	Detail
Company size	500–10,000+ employees
Revenue	$100M+ ARR
Ops team	200–2,000+ staff
Daily task volume	10,000–100,000+ portal interactions per day
Pain intensity	Prior auth denial management alone costs millions annually
Buying persona	VP Revenue Cycle, Chief Operating Officer, SVP Operations
Deal size	$50K–$200K+/month
Sales cycle	3–6 months (security review, pilot, rollout)

Tertiary ICP: Digital Health Startups

Attribute	Detail
Company size	5–50 employees
Funding stage	Seed through Series B
Engineering team	3–15 engineers
Core need	Legacy system integration as part of product
Buying persona	CEO, CTO, CPO, Founding Engineer
Deal size	$3K–$10K/month
Sales cycle	1–2 weeks (fast movers)

5. High-Value Use Cases

Use Case 1: Prior Authorization Automation

Current Manual Process: A staff member logs into a payer portal, navigates to the prior auth section, manually enters patient demographics, diagnosis and procedure codes, clinical justification, and uploads supporting documents (lab results, physician notes, imaging reports). They submit the request, then check back repeatedly—often daily—for approval status. Each submission takes 15–45 minutes. Staff must learn and navigate 10–50+ different portal interfaces, each with unique layouts, field names, and submission rules.

Key Inefficiencies: Average 20–30 minutes per submission across portals. High error rates (wrong codes, missing documents) leading to 10–25% denial rates. Status checks require repeated manual logins. Portal UI changes break any existing macros or scripts. Staff training costs for each new portal.

How Asteroid Replaces/Augments: Browser agents log into any payer portal, auto-fill all required fields from structured data, upload clinical documents, submit the request, and poll for status updates. Self-healing agents adapt when portal UIs change. Full audit trail for every submission. Human review flagged only for exceptions.

Estimated Savings: Reduces per-submission time from 20–30 minutes to 2–3 minutes of agent execution. At 500 submissions/day, this saves approximately 150 FTE-hours daily. At $20/hr loaded cost, that represents $3,000/day or $780,000/year in direct labor savings for a single mid-sized organization.

Use Case 2: Coverage Verification & Eligibility Checks

Current Manual Process: Staff log into individual payer portals (Highmark, Kaiser, BCBS, IBX, Sentara, CareFirst, HealthPartners, Jefferson Health, Medica, Horizon, and others), enter patient identifiers, and manually check coverage status, benefits, copay amounts, and eligibility dates. Results are copied into the organization's internal system.

Key Inefficiencies: Each check takes 5–10 minutes per portal. Staff must maintain login credentials for dozens of portals. No standardized output format across payers. Manual data re-entry creates errors. Batch processing is impossible—each check is sequential.

How Asteroid Replaces/Augments: Browser agents execute coverage checks across all target payer portals in parallel. Structured results returned via API for direct ingestion into internal systems. Vitable Health demonstrates this exact use case, building agents independently across 10+ carrier portals.

Estimated Savings: Reduces per-check time from 5–10 minutes to under 60 seconds. At 1,000 checks/day, saves approximately 80–160 FTE-hours daily. Annual savings of $400K–$800K for a mid-sized organization.

Use Case 3: Claims Status Checks

Current Manual Process: Revenue cycle staff log into payer portals to check the status of submitted claims—whether they're processing, approved, denied, or require additional information. Each claim requires navigating to the correct portal, entering the claim number or patient information, and recording the result.

Key Inefficiencies: Organizations may need to check status on thousands of claims daily across dozens of portals. Each check takes 3–5 minutes. Results must be manually entered into the billing system. Delays in identifying denials lead to missed appeal windows.

How Asteroid Replaces/Augments: Agents perform batch claims status checks across all relevant payer portals, returning structured results (status, denial codes, required actions) via API. Proactive alerts when denials are detected, enabling immediate action within appeal windows.

Estimated Savings: At 2,000 claims/day, reduces status checking from 100–170 FTE-hours to near-zero manual effort. Faster denial detection can recover 5–15% of previously lost revenue from missed appeals.

Use Case 4: EHR Data Entry & Retrieval

Current Manual Process: Clinical and administrative staff manually log into EHR systems to enter patient data (demographics, clinical notes, lab results, referral information) or retrieve existing records. Many legacy EHRs, especially smaller or older systems, lack APIs entirely. Some are only accessible through Citrix virtual desktops.

Key Inefficiencies: Manual data entry is slow (10–20 minutes per patient record), error-prone, and requires trained staff familiar with each EHR's interface. Organizations working with multiple EHRs face exponential complexity. Citrix-based systems are particularly challenging to automate.

How Asteroid Replaces/Augments: Browser agents handle web-based EHRs; computer-use agents (CUA) handle Citrix/desktop-based systems. Moonset Health demonstrates this at scale—CUA agents navigate 10+ legacy hospice EMR systems via Citrix across the US. Zyter is evaluating Asteroid for connecting to fragmented smaller EHRs.

Estimated Savings: Reduces per-record data entry from 10–20 minutes to 1–3 minutes. For organizations managing thousands of patient records across multiple EHRs, this represents millions in annual labor savings and dramatically reduced error rates.

Use Case 5: Clinical Trial Enrollment Automation

Current Manual Process: After a voice agent completes a pre-screening call with a clinical trial candidate, staff must manually log into the clinical management system (CTMS), navigate to the patient profile, and fill out a multi-field screening questionnaire based on the call transcript. This involves interpreting unstructured conversation data and mapping it to structured form fields.

Key Inefficiencies: Manual form completion takes 15–30 minutes per patient. Transcription errors lead to screening failures. CTMS platforms are legacy systems with no APIs. Clinical research coordinators are expensive ($60K–$90K/year) and in short supply.

How Asteroid Replaces/Augments: Delfa's voice agent passes call transcripts to Asteroid browser agents via API. The agent logs into the CTMS, navigates to the patient profile, parses

the transcript, and fills out the entire screening questionnaire. Exceptions are flagged for human review. Full HIPAA-compliant audit trail.

Estimated Savings: Delfa achieved 0% to 100% form completion rate, 95% engineering effort saved, and went from zero to production in 30 minutes. Per-patient enrollment processing drops from 15–30 minutes to under 2 minutes.

Use Case 6: Fax & Document Workflows

Current Manual Process: Despite the digital age, healthcare still runs on fax. Staff must log into fax platforms (like Documo), upload documents, enter recipient information, send faxes, and then confirm delivery. Inbound faxes must be reviewed, classified, and routed to the correct department or patient record.

Key Inefficiencies: Manual fax workflows take 5–15 minutes per document. High volume (100s–1,000s/day at scale). Errors in routing lead to compliance violations. Staff time is disproportionately spent on a low-value task.

How Asteroid Replaces/Augments: Browser agents automate fax sending, document uploads, and delivery confirmation across platforms like Documo. Thyme Care uses this in production with 1,000s of daily executions, demonstrating reliability at scale.

Estimated Savings: Reduces per-fax processing from 5–15 minutes to under 1 minute. At 500 faxes/day, saves 40–120 FTE-hours daily.

Use Case 7: Appointment Booking (Voice-to-Browser)

Current Manual Process: A patient calls to book an appointment. A receptionist answers, collects information, then navigates the practice management system to find available slots and book the appointment. In the UK, this involves NHS systems with complex booking interfaces.

Key Inefficiencies: Phone lines are only available during business hours. Receptionists handle 50–100 calls/day. Complex booking systems slow down the process. Missed calls mean lost appointments and revenue.

How Asteroid Replaces/Augments: An AI voice agent handles the patient call 24/7, collecting all necessary information. The call transcript is automatically passed to an Asteroid browser agent, which books the appointment in the practice management system. InTouchNow demonstrates this pipeline for GP practices in the UK.

Estimated Savings: Enables 24/7 appointment booking without receptionist staff. Each voice-to-browser booking takes 1–2 minutes vs. 5–10 minutes manually. Eliminates missed calls and after-hours booking gaps.

6. Competitive Positioning

Asteroid competes across four categories: traditional RPA, API-based integration platforms, in-house automation, and emerging AI browser agent competitors. In healthcare specifically, Asteroid holds decisive advantages in each comparison.

Dimension	Asteroid.ai	Traditional RPA (UiPath, AA)	API Platforms (Mulesoft, etc.)	In-House Automation
API requirement	None—works via browser/screen	None (screen scraping), but fragile	APIs required (deal-breaker in healthcare)	Varies—usually requires APIs or custom scripts
Setup time	Minutes (Astro no-code builder)	Weeks to months	Months (per integration)	Months (engineering sprints)
Portal change resilience	Self-healing agents adapt	Breaks—requires manual script updates	N/A (API dependent)	Breaks—requires re-engineering
Citrix/Desktop support	Yes (CUA—first in healthcare)	Yes (core strength), but complex setup	No	Rarely—requires specialized tooling
HIPAA compliance	Built-in audit trails, encrypted	Possible but requires configuration	Depends on implementation	Must be built from scratch
Healthcare specificity	Purpose-built for healthcare workflows	Generic—healthcare is one of many verticals	Generic middleware	Custom to organization
Maintenance burden	Low—self-healing, managed	High—scripts break frequently	Medium—API versioning issues	Very high—ongoing engineering cost
Cost model	$3K–$50K+/mo usage-based	$50K–$500K+/year license + implementation	$100K+/year platform + per-integration costs	2–5 FTEs ($200K–$750K/year)

Why Asteroid Wins in Healthcare

- **The API gap is permanent:** Healthcare systems were built decades ago. The 1,000+ EHR vendors and hundreds of payer portals will not build APIs. Any solution that requires APIs is structurally locked out of most of the market. Asteroid is built for this reality.

- **Portal fragility kills RPA:** Traditional RPA records screen coordinates, element IDs, and pixel positions. When payer portals update their UI (which they do frequently and without notice), RPA scripts break. Asteroid's self-healing agents use AI to understand interface context, not brittle selectors.
- **Speed to value is a competitive weapon:** Healthcare ops leaders are used to 6–12 month integration timelines. Telling them "30 minutes to production" (Delfa) or "self-serve with no engineering" (Vitable) is disruptive and memorable. This compresses sales cycles dramatically.
- **Citrix/CUA is a moat:** Moonset Health's deployment on legacy hospice EMRs via Citrix is something no competitor has demonstrated in healthcare. This opens the hospice, home health, long-term care, and behavioral health segments—all of which run on legacy desktop systems.

7. Messaging Strategy

The following positioning angles are designed for consultative, mid-market and enterprise outreach. Each angle targets a specific pain point and buyer persona.

Angle 1: “Automate any healthcare workflow—no APIs needed”

Target Audience: CTOs, VP Engineering, Product Leaders at healthtechs

Pain Point: Engineering teams waste months building and maintaining custom integrations with legacy systems that lack APIs.

Positioning & Outcome: Position Asteroid as the integration layer that replaces 6 months of engineering with 30 minutes of agent setup. Lead with the Delfa case study (95% engineering effort saved).

Angle 2: “Turn 1,000s of manual portal tasks into autonomous workflows”

Target Audience: Head of Operations, VP Ops at healthcare ops companies

Pain Point: Large ops teams spending 80% of their time on repetitive browser-based tasks across dozens of portals.

Positioning & Outcome: Position Asteroid as the force multiplier for ops teams. Lead with Thyme Care (1,000s of daily executions). Quantify: “What would your ops team accomplish if they didn’t spend 6 hours/day on portal work?”

Angle 3: “Fix broken prior auth workflows in days, not months”

Target Audience: Rev Cycle Managers, VP Clinical Operations, Heads of Operations

Pain Point: Prior auth takes 15–45 minutes per submission. Denial rates of 10–25%. Portals change and break automation.

Positioning & Outcome: Position Asteroid as the only solution that works across any payer portal without APIs, adapts to portal changes, and provides a full audit trail. Quantify labor savings based on volume.

Angle 4: “Automate what legacy systems won’t let you integrate”

Target Audience: CTOs, CIOs, IT Leaders at health systems and enterprise healthtechs

Pain Point: Legacy EHRs and Citrix-based systems have no APIs, no integrations, no modern access. Manual workarounds are the only option.

Positioning & Outcome: Position Asteroid's CUA capability as the breakthrough. Lead with Moonset Health (10+ hospice EMR systems via Citrix). Emphasize: "If a human can do it in a browser or Citrix session, our agents can too."

Angle 5: "Stop scaling headcount. Start scaling automation."

Target Audience: COO, VP Ops, CFO at mid-market and enterprise healthcare organizations

Pain Point: Every 2x growth in volume requires 2x growth in ops headcount. Labor costs are 30–60% of operating expenses.

Positioning & Outcome: Position Asteroid as the way to decouple growth from headcount. Quantify: one Asteroid deployment can handle the work of 5–20 FTEs. Frame as unit economics improvement, not just cost cutting.

Angle 6: "HIPAA-compliant automation with full audit trails"

Target Audience: Compliance Officers, VP Ops, Legal/Risk leaders

Pain Point: Automation tools that don't meet HIPAA requirements are non-starters. In-house solutions lack proper audit trails.

Positioning & Outcome: Lead with compliance as a differentiator, not an afterthought. Every Asteroid execution has a full audit trail. HIPAA compliance is built into the infrastructure, not bolted on.

Angle 7: "The last-mile automation for your voice AI platform"

Target Audience: CEOs, CTOs at AI voice agent startups

Pain Point: Voice agent handles the call, but someone still has to manually enter data into the EHR/CTMS/booking system.

Positioning & Outcome: Position Asteroid as the browser execution layer that completes the voice agent pipeline. Lead with InTouchNow (GP booking) and Delfa (clinical trial enrollment). "Your voice AI handles the conversation. Our browser AI handles the system."

8. Outreach Playbook

This playbook is designed for a consultative sales motion targeting mid-market and enterprise healthcare organizations. The approach leads with the prospect's pain, positions Asteroid as a strategic partner, and uses social proof from existing customers to build credibility.

Channel Strategy

LinkedIn (Primary Channel)

Strategy: Personalized connection requests and DMs targeting Heads of Ops, VPs, Rev Cycle Managers, and CTOs at target accounts. Lead with a specific observation about their company, reference a relevant pain point, and offer value (case study, industry insight) before asking for time.

Personalization approach: Research the company's job postings (ops roles indicate manual workload), recent funding (scale = more volume), tech stack mentions (legacy EHRs, specific payer portals), and LinkedIn posts by the target persona (complaints about manual work, hiring challenges).

5 LinkedIn DM Openers

- "I noticed [Company] is hiring for [X] ops roles — that's usually a sign the team is drowning in portal work. We helped a similar company (oncology navigation, Series C) automate 1,000s of daily portal tasks and reallocate their ops team to higher-value work. Worth a 15-min conversation?"
- "Congrats on the [Series X] raise. As you scale, the manual prior auth workload is going to scale linearly with you — unless you automate it. We're working with several healthtechs on exactly this. Happy to share what we're seeing if useful."
- "Saw your team works across [specific payer portals]. We built an automation layer that handles coverage verification across Highmark, Kaiser, BCBS, and 10+ other portals without needing any APIs from them. Curious if this pain resonates?"
- "Quick question: how much of your ops team's day is spent on manual browser work — logging into portals, filling forms, checking statuses? We've been helping healthtechs reclaim that time. Would love to share a relevant case study."
- "I saw [Company] is building [product for healthcare]. When it comes to integrating with legacy EHRs and payer portals, we've found that 90% of them have no APIs. We help companies like yours automate that 'last mile' of system interaction. Worth chatting?"

Email (Secondary Channel)

Strategy: Cold outreach using enrichment tools (Apollo/Clay) for targeting and sequencing tools (Instantly or similar) for delivery. Healthcare-specific messaging. 3–4 touch sequence: value-first email, case study follow-up, brief check-in, breakup email.

Personalization approach: Reference the prospect's specific EHR/payer mix, recent operational hires, public statements about scaling challenges, or industry pain points. Never send generic automation messaging.

5 Cold Email Hooks

- Subject: "Your ops team shouldn't be the bottleneck" — "Hi [Name], I noticed [Company] is processing [X volume] of prior auths across multiple payer portals. Our browser AI agents can automate 80%+ of that workflow—without needing a single API from those portals. We did this for an oncology navigation company running 1,000s of daily operations. Worth 15 minutes?"
- Subject: "Re: [Payer portal] automation" — "Hi [Name], we're seeing a pattern: healthtechs spending $500K+/year on staff to manually log into payer portals and check coverage. We built agents that do this across Highmark, Kaiser, BCBS, and dozens more. A direct care startup reduced their verification time by 90%. Relevant for [Company]?"
- Subject: "30 minutes to automate your legacy EHR integration" — "Hi [Name], most healthtechs spend 3–6 months building integrations with legacy EHRs that lack APIs. We helped a clinical trial platform go from zero to production in 30 minutes—automating patient enrollment across legacy CTMS systems. If legacy integration is eating your engineering bandwidth, I'd love to show you how."
- Subject: "You're hiring 10 ops people. What if you needed 2?" — "Hi [Name], saw [Company] has [X] open ops roles. In our experience, 60–80% of healthcare ops work is repetitive browser tasks that AI agents can handle. We helped a healthcare ops company automate their portal updates, fax workflows, and data entry—freeing their team for work that actually requires judgment. Open to exploring this?"
- Subject: "Citrix and legacy EMRs don't have to be manual" — "Hi [Name], we deployed the first AI agents that operate inside Citrix environments in healthcare—navigating legacy hospice EMRs that have no web interface, no API, and no integration path. If your team is manually operating in legacy desktop systems, we should talk."

Warm Intros (Highest Conversion)

Strategy: Leverage YC network (David's connections), existing customer referrals, and investor intros. Frame as: "I'm working with [reference customer] on [specific use case]. [Mutual connection] suggested you might be facing a similar challenge."

- Ask Thyme Care's Head of Ops (Jonathan) for intros to peers at other oncology/specialty care companies.
- Use YC Bookface to identify healthcare founders in the network and request warm intros.
- After closing each new customer, systematically ask for 2–3 referrals to similar companies.

9. Target Account Examples

The following are types of companies that represent high-probability targets based on Asteroid's validated use cases and ICP criteria. These are representative profiles, not exhaustive lists.

Account Type	Characteristics	Primary Use Case	Why They Fit	Est. Deal Size
Mid-market RCM companies	100–500 employees, $10M–$50M revenue, large ops teams	Prior auth + claims status	Process 1,000s of prior auths and claims daily across dozens of payer portals; manual labor is primary cost driver	$10K–$30K/mo
Oncology navigation companies	50–250 employees, venture-backed, complex care coordination	Portal updates + fax workflows	Similar to Thyme Care; navigate multiple payer portals and fax systems daily at high volume	$6K–$15K/mo
Direct primary care startups	20–100 employees, Series A–B, multi-payer	Coverage verification	Similar to Vitable; need to verify coverage across 10+ carrier portals for every patient	$3K–$10K/mo
AI voice agent companies (healthcare)	10–50 employees, seed–Series B, building for healthcare	Voice-to-browser pipeline	Similar to InTouchNow/Delfa; voice agent handles calls but needs browser automation for system interaction	$3K–$10K/mo
Hospice & home health companies	100–500 employees, legacy Citrix-based systems	EHR data entry via CUA	Similar to Moonset; operate on legacy EMRs accessible only through Citrix/desktop; no other automation option exists	$6K–$20K/mo
Clinical trial CROs & site networks	50–500 employees, legacy CTMS platforms	Trial enrollment automation	Similar to Delfa; CTMS platforms lack APIs; manual patient enrollment is bottleneck for trial timelines	$6K–$15K/mo
Specialty pharmacy operations	100–300 employees, high prior auth volume	Prior auth + coverage verification	Specialty drugs require prior auth on nearly every prescription; 100s of	$10K–$30K/mo

Account Type	Characteristics	Primary Use Case	Why They Fit	Est. Deal Size
			daily submissions across payer portals	
Health plan BPO providers	500–2,000+ employees, process outsourcing	Claims + eligibility + prior auth	Entire business model is processing portal work at scale; automation directly improves margins	$30K–$100K+/mo
Dental & vision benefits administrators	50–200 employees, multi-carrier portal work	Eligibility checks + claims	Must verify benefits across dozens of dental/vision carrier portals with no APIs	$6K–$15K/mo
Enterprise health systems (500+ beds)	1,000+ employees, revenue cycle departments	Prior auth + claims at scale	Internal rev cycle teams process 5,000–20,000+ portal interactions daily; even small efficiency gains save millions	$50K–$200K/mo

10. Final Strategic Recommendations

Immediate Actions (Next 30 Days)

- **1. Accelerate prior auth outbound:** The 30-lead campaign already underway is Asteroid's fastest path to validated mid-market revenue. Expand to 100+ leads. Target: RCM companies, specialty pharmacies, and health system rev cycle departments. Goal: 10–15 qualified demos.
- **2. Build the Thyme Care case study:** Thyme Care ($82M raised, 1,000s of daily executions) is Asteroid's most powerful credibility asset for mid-market and enterprise healthcare ops. Create a detailed, quantified case study with Jonathan O'Donnell's endorsement. This becomes the anchor asset for every enterprise conversation.
- **3. Productize the payer portal play:** Vitable's success across 10+ carrier portals is a repeatable, horizontal use case. Build a landing page and one-pager specifically for payer portal automation, listing supported portals (Highmark, Kaiser, BCBS, IBX, Sentara, CareFirst, HealthPartners, Jefferson Health, Medica, Horizon). This becomes a searchable, inbound-generating asset.
- **4. Launch the Tennr follow-up:** The prior auth demo is already prepared. Follow up immediately, as Tennr represents both a customer opportunity and a validation of the prior auth use case with a well-known brand.

Medium-Term Actions (30–90 Days)

- **5. Develop Citrix/CUA positioning for hospice & home health:** Moonset Health's deployment is a genuine moat—no competitor has demonstrated healthcare CUA in production. Build dedicated content (blog post, one-pager, demo video) targeting hospice, home health, and long-term care companies. These organizations have zero automation alternatives.
- **6. Create the voice-to-browser playbook:** AI voice agent companies are a growing sub-segment with a clear, repeatable need. Publish content (blog, case study with InTouchNow and Delfa) positioning Asteroid as the standard browser execution layer for healthcare voice AI. Target: AI voice companies building for healthcare scheduling, intake, and clinical trials.
- **7. Build a vertical content engine:** Publish one healthcare-specific blog post per week targeting high-intent search terms: "payer portal automation," "prior auth automation," "legacy EHR integration," "Citrix healthcare automation." Each post should reference specific use cases and include a clear CTA for demo booking.

Strategic Positioning Principles

- **Lead with the prospect's pain, not the product:** Every outreach message should start with a specific, quantifiable operational challenge—not with what Asteroid does. The prospect should feel understood before they hear the solution.
- **Sell speed to value:** Asteroid's fastest path to differentiation is deployment speed. "30 minutes to production" and "self-serve with no engineering" are disruptive claims in

healthcare, where 6–12 month timelines are the norm. Make this a headline in every conversation.

- **Use reference customers as proof, not logos:** In healthcare, trust is earned through specific, verifiable outcomes. Reference Thyme Care's 1,000s of daily executions, Delfa's 95% engineering savings, Vitable's self-serve portal automation, and Moonset's Citrix deployment. Outcomes are more powerful than brand names.
- **Position compliance as a feature, not a checkbox:** Every competitor can claim HIPAA compliance. Asteroid should demonstrate it—full audit trails on every execution, encrypted infrastructure, human-in-the-loop governance. Make compliance a selling point, not a FAQ item.
- **Build the category, not just the pipeline:** Asteroid is not just another automation tool. It is defining a new category: AI browser agents for healthcare. Every piece of content, every sales conversation, and every case study should reinforce this positioning. Category creators capture disproportionate market share.

End of Report — Prepared for Asteroid.ai Sales & Marketing Team